

CRLN-TRG-001

CRLN Clinical Research Competency Framework — Version 1.7

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***Maturity notice:** CRLN-TRG-001 is an open, versioned competency framework operationalizing recognized standards. The domain taxonomy and crosswalk are published openly; the standards-crosswalk mappings are undergoing subject-matter-expert (SME) review and are marked provisional until signed off. This is a transparent, evolving standard — published openly so the field can inspect, cite, and build on it.*

1. Purpose & scope

CRLN-TRG-001 defines a common, openly-published competency framework for the global clinical-research workforce. Its purpose is to make clinical-research competency **measurable, comparable, and portable** across countries, languages, and roles — anchored to the recognized global standard (ICH E6(R3) Good Clinical Practice) and aligned to the industry-recognized Joint Task Force (JTF) Clinical Trial Competency Framework. It defines **fourteen role tracks** and is delivered free to learners worldwide:

role	track
CRC	Clinical Research Coordinator
CRA	Clinical Research Associate
CTA	Clinical Trial Administrator
CDM	Clinical Data Manager
PM	Clinical Trial / Project Manager
RA	Regulatory Affairs Specialist
PV	Pharmacovigilance / Drug Safety Associate
PI	Principal Investigator
SUBI	Sub-Investigator
RN	Research Nurse
PHARM	Clinical Trial Pharmacist
DCT	Decentralized Trial Coordinator
ASSIST	Research Assistant / Study Coordinator I

role	track
LAB	Specimen Processing / Laboratory Technician

2. Canonical competency taxonomy (10 domains)

CRLN-TRG-001 organizes clinical-research competency into ten canonical domains (D-01...D-10), aligned to the ICH E6(R3) structure:

ID	Domain
D-01	GCP Principles & Ethics
D-02	Informed Consent Processes
D-03	Protocol Adherence & Deviations
D-04	Adverse Event & Safety Reporting
D-05	Investigational Product Management
D-06	Data Integrity & Source Documentation
D-07	IRB / IEC Submissions
D-08	Trial Master File & Regulatory Documentation
D-09	Site Oversight & Monitoring
D-10	Pharmacovigilance & Regulatory Submissions

(As of version 1.7 the framework comprises **84 competency statements** — fourteen role tracks by six domains each — across **8 published versions** (1.0 through 1.7). Domain codes D1–D6 are positional within a role: D1 for a CRA and D1 for a Clinical Data Manager are different competencies, so comparisons must use the fully-qualified id CRLN- <ROLE> - <Dn>. Full item-level detail is maintained in the CRLN methodology registry.)

*Correction, 2026-08-31: this paragraph previously read "56 competency statements ... and 3 tracked framework versions." Both figures were produced by counting the methodology registry without filtering, so they included the eight JTF v3.1 reference statements that CRLN-TRG-001 is crosswalked **to** rather than composed of. The framework's own content is 48 statements across 2 versions.*

3. Standards crosswalk — CRLN-TRG-001 ↔ JTF ↔ ICH E6(R3)

CRLN operationalizes recognized standards rather than inventing its own. Each canonical domain maps to the Joint Task Force Clinical Trial Competency Framework and to ICH E6(R3), with ACRP/SOCRA relevance noted. Section references use **ICH E6(R3) Annex 1** numbering throughout, taken from the adopted guideline. (Mappings provisional pending SME review.)

Correction, 2026-09-01 (release 1.1.3): until this release, 38 of the 48 competency domains cited ICH E6(R2) section numbers — §4 for the Investigator, §5 for the Sponsor, §8 for Essential Documents — under an "ICH E6(R3)" label. R3 Annex 1 places those at §2, §3 and Appendix C. The mappings were right; the addresses were wrong. They now use verified R3 Annex 1 anchors.

CRLN	Domain	JTF domain	ICH E6(R3) area	ACRP/SOCRA relevance
D-01	GCP Principles & Ethics	4 · Clinical Study Operations (GCPs)	Principles of ICH GCP	GCP foundations
D-02	Informed Consent Processes	2 · Ethical & Participant Safety	Informed consent of trial participants	Informed consent
D-03	Protocol Adherence & Deviations	4 · Clinical Study Operations (GCPs)	Investigator compliance with protocol	Protocol compliance & deviations
D-04	Adverse Event & Safety Reporting	2 · Ethical & Participant Safety	Safety reporting (investigator & sponsor)	Safety reporting
D-05	Investigational Product Management	3 · Investigational Products Dev & Reg	IP management & accountability	IP accountability
D-06	Data Integrity & Source Documentation	6 · Data Management & Informatics	Records, source data & ALCOA+	Data integrity, 21 CFR Part 11
D-07	IRB / IEC Submissions	2 · Ethical & Participant Safety	IRB/IEC responsibilities & submissions	IRB/IEC
D-08	Trial Master File & Regulatory Documentation	4 · Clinical Study Operations (GCPs)	Essential records (TMF)	TMF / essential documents
D-09	Site Oversight & Monitoring	5 · Study & Site Management	Monitoring & risk-based quality management	Monitoring, SDV/SDR, RBM
D-10	Pharmacovigilance & Regulatory Submissions	3 · Investigational Products Dev & Reg	Sponsor safety & regulatory reporting	PV & regulatory submissions

4. Regulatory jurisdiction coverage

The framework is mapped to national regulatory regimes to ensure GCP applicability across markets. Current coverage (expanding):

Region	Country	Authority	ICH status	GCP basis
North America	United States	FDA	Founding regulatory member	ICH E6 GCP
North America	Canada	Health Canada	Member	ICH E6 GCP
Europe	European Union	EMA + national competent authorities	Founding regulatory member	ICH E6 GCP
Europe	United Kingdom	MHRA	Member	ICH E6 GCP
Europe	Switzerland	Swissmedic	Member	ICH E6 GCP
Asia-Pacific	Japan	PMDA / MHLW	Founding regulatory member	ICH E6 GCP
Asia-Pacific	China	NMPA	Member	ICH E6 GCP
Asia-Pacific	South Korea	MFDS	Member	ICH E6 GCP

Region	Country	Authority	ICH status	GCP basis
Asia-Pacific	Singapore	HSA	Member	ICH E6 GCP
Asia-Pacific	Australia	TGA	Observer	ICH E6 GCP
Asia-Pacific	Philippines	FDA Philippines	Adopter	ICH E6 GCP
South Asia	India	CDSCO	Observer	ICH E6 GCP (adapted)
South Asia	Pakistan	DRAP	Adopter	ICH E6 GCP
Africa	South Africa	SAHPRA	Observer	ICH E6 GCP
Africa	Kenya	PPB	Observer/adopter	ICH E6 GCP
Africa	Nigeria	NAFDAC	Adopter	ICH E6 GCP
Latin America	Brazil	ANVISA	Member	ICH E6 GCP
Latin America	Mexico	COFEPRIS	Adopter	ICH E6 GCP
Latin America	Argentina	ANMAT	Adopter	ICH E6 GCP
Middle East	Türkiye	TITCK	Observer	ICH E6 GCP

5. Methodology (summary)

- **Standard anchor:** ICH E6(R3) Good Clinical Practice; alignment to the JTF Clinical Trial Competency Framework.
- **Measurement:** scenario-based assessment scored against a published rubric; item-response-theory (IRT/Rasch) ability estimation (engine proprietary).
- **Privacy:** no PHI; pseudonymous identifiers; k-anonymized aggregates (minimum cohort 5); consent-aware.
- **Citations:** 24 standard citations underpin the framework (ICH, regulatory, and competency-framework sources).
- **Validation status (transparent):** The **domain crosswalk is independently validated**. Three subject matter experts from three organisations each reviewed all ten domain mappings, with unanimous agreement and no divergent or contested mapping (10 of 10 validated, 0 divergent, 0 contested, as at 2026-09-18). Reviewers declared their affiliations and any conflict of interest; the review record is held against the instrument version.

Item-level sign-off is NOT complete and is not claimed. No assessment item has yet reached the three-rater consensus threshold. Empirical psychometric validation (calibration, reliability, cross-language invariance) remains the subject of ongoing research. Item-level claims are marked directional until validated.

The distinction is deliberate and load-bearing. The crosswalk establishes that CRLN's competency domains map correctly onto the external framework; it does not establish that any individual item measures what it claims. Those are different assertions and only the first is currently supported.

6. Governance & change control

Versioned and openly published. Changes are tracked in the CRLN methodology registry with change notes; published versions are immutable. A Methodology Review Board, conflict-of-interest policy, and appeals process govern updates. Each published version receives its own DOI.

7. How to cite

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